Resolution Resistance

A Structural Model of Why
Regulation Architectures Resist Change

Jani Halmetoja

Contact: halmetojamodel@gmail.com

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Abstract

The Structural Regulation Framework (SRF; Halmetoja, 2026) models the architecture of psychological regulation — where coherence is maintained, how quickly activation externalizes, and through which channels. What it does not address is why regulation architectures resist modification, what occurs during parameter transition, and what structural conditions enable sustainable change.

This paper proposes that regulation architectures exhibit predictable resistance to modification that is not reducible to motivational deficit, cognitive limitation, or therapeutic failure. The resistance is structurally determined by the same parameters the framework describes.

The paper introduces a primary mechanism — Resolution Resistance — proposing that highly compressed dichotomous processing persists because it minimizes destabilization risk under insufficient holding capacity. From this primary mechanism, three derived predictions follow: (1) Transitional Pain — increasing psychological resolution initially amplifies distress through decompression of previously simplified experience; (2) Scaffolded Integration — sustainable change requires temporary external holding structures compensating for insufficient internal capacity during destabilization; (3) Domain-Selective Resolution — processing resolution operates domain-specifically, with defensive compression maintained preferentially in high-fragmentation-risk domains.

The paper formalizes the bootstrap problem inherent in structural change, proposes a heuristic attractor basin interpretation of regulatory stability, distinguishes forced from scaffolded resolution increase, and derives testable predictions with explicit falsification criteria.

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1. Introduction

1.1 The Missing Model

The SRF (Halmetoja, 2026) describes regulatory states — the IRC × SF space, externalization vectors, REL dynamics — but treats movement between states as an outcome rather than a process. It predicts that therapy may increase IRC and SF (SRF H5, H7) without modeling the mechanism, trajectory, or structural preconditions of that increase.

This gap matters because the most clinically significant question about any regulation architecture is not "what is it?" but "can it change, and under what conditions?"

1.2 Why Architectures Persist

Clinical observation consistently demonstrates that externalized regulation architectures resist modification with remarkable tenacity. This is particularly visible in configurations organized around narcissistic dynamics, but the phenomenon is not limited to them. Standard explanations invoke motivational deficit ("they don't want to change"), cognitive limitation ("they can't see it"), or characterological fixity ("personality is stable").

This paper proposes a structural alternative: the architecture persists because modification would require capacities the architecture itself prevents from developing. This is a formal conceptual model of parameter interdependence, intended to generate testable hypotheses about change resistance.

1.3 Scope and Relationship to SRF

This paper assumes familiarity with SRF constructs (IRC, SF, ERD, REL, externalization vectors, the IRC × SF space). It extends the framework by modeling dynamics — how systems move (or fail to move) through the IRC × SF space over time. The framework is intentionally integrative rather than school-specific — drawing on psychodynamic, attachment, systems, and affect regulation traditions to the extent that each contributes structural specificity to the change dynamics model.

Terminological Clarifications

IRC in this paper is conceptualized as a dynamic holding-capacity parameter — state-modifiable but structurally constrained. It is not a fixed trait (it can develop through experience) nor a momentary state (it has architectural stability). IRC describes the system's current maximum capacity to contain unresolved activation without discharge. This capacity can change over developmental time but is not freely variable moment-to-moment.

Resolution in this paper refers to the observable effective expression of SF under current activation conditions — the degree of experiential complexity the system is currently representing. Resolution is not a separate construct from SF but rather SF's functional output in a given context. A system with moderate SF capacity may express high resolution in low-activation domains and low resolution in high-activation domains (see Section 7).

Destabilization in this paper refers to the disruption of coherent self-state organization — operationally manifesting as identity diffusion, emotional flooding, loss of stable self-representation, or inability to maintain consistent self-narrative. It does not refer to psychotic disorganization. The severity ranges from temporary incoherence (recoverable within hours) to acute identity crisis (requiring days to weeks for restabilization).

The paper does not repeat SRF definitions. Readers unfamiliar with the framework should consult Halmetoja (2026) for foundational constructs.

Construct Mapping

To situate SRF constructs relative to existing empirical literature:

SRF construct	Approximate empirical neighbors	Distinction
IRC	Distress tolerance / ego strength / affect tolerance	Threshold framing
SF	Mentalization / cognitive flexibility / emotional differentiation	Contradiction holding
ERD	Attachment dependence / reassurance seeking	Regulatory dependence
REL	Response latency / impulsivity	Externalization timing

These are not equivalences. Each SRF construct overlaps with but is not reducible to its empirical neighbors. The "distinction" column identifies the specific aspect the SRF construct emphasizes that existing measures may not fully capture. In particular: existing constructs typically describe regulatory capacity dimensionally (more or less of a continuous trait), whereas IRC is explicitly framed as a threshold parameter governing transition between stability and destabilization under increasing processing demand. This threshold framing generates predictions about discontinuous change (the J-curve, the bootstrap problem) that dimensional capacity models do not naturally produce.

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2. Structural Flexibility as Active Protection

2.1 The Standard View

The SRF describes low SF through its failure modes: splitting, simplification, denial, rigidity. This framing implies deficit — something missing, something broken. The standard clinical interpretation follows: the patient lacks flexibility, needs to develop it, and therapeutic interventions should target this deficit.

2.2 The Reframe

Low SF — particularly low contradiction holding and low reality contact — may function not merely as a deficit but as an active protective mechanism that minimizes destabilization risk under insufficient IRC.

Highly compressed dichotomous processing reduces the complexity of internal experience to a level the system can structurally contain. A system operating at binary resolution does not encounter:

- **Contradiction**: self as simultaneously good and harmful
- **Shame**: self as having caused irreversible damage
- **Dependency**: self as requiring others for coherence
- **Grief**: self as having lost what cannot be recovered

Each of these experiences requires IRC to contain. A system with insufficient IRC cannot hold these experiences without destabilization — acute self-state fragmentation, identity incoherence, or disorganizing affect overload.

Low SF is therefore not merely the absence of flexibility. It is the presence of compression — an active reduction of experiential bandwidth to match available containment capacity.

2.3 The Signal Processing Analogy

Drawing on the A/D converter model (Halmetoja, 2026, artwork series), SF can be heuristically conceptualized through a signal processing analogy: effective bit depth of psychological processing — the number of distinguishable gradations the system can represent between experiential extremes. This analogy is offered as an explanatory tool, not an ontological claim about cognitive architecture.

Effective resolution	Self-representation	Other-representation	Structural requirement
Binary	"I am excellent"	"They are wrong"	Minimal IRC
Low-resolution	"I am mostly competent"	"They may have a point"	Low-moderate IRC

Effective resolution	Self-representation	Other-representation	Structural requirement
Moderate-resolution	"I am partly good, partly inadequate"	"They are partly right, and I have caused harm"	Substantial IRC
High-resolution	Full ambivalence, responsibility, dependency, grief, and self-compassion held simultaneously	Full complexity of other without splitting	High IRC + high SF

Higher resolution is more accurate. But accuracy has structural costs. The system must be able to contain what higher resolution reveals.

2.4 Functional Implication

If low SF is protective rather than merely deficient, then interventions targeting SF directly (confrontation, interpretation, reality-testing) without first establishing adequate IRC are not merely ineffective — they are structurally destabilizing. They attempt to increase resolution beyond the system's containment capacity.

This reframes therapeutic resistance from opposition to self-preservation.

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3. The Resolution Resistance Hypothesis

3.1 Formal Statement

H1 — Resolution Resistance: Highly compressed dichotomous processing persists not solely because it is cognitively preferred, but because it minimizes destabilization risk under insufficient structural holding capacity.

3.2 Mechanism

For a system with IRC below a critical threshold (IRC_crit), maintaining low SF is proposed to be structurally adaptive:

1. Low SF compresses experiential complexity to containable levels
2. The compressed representation is coherent (though inaccurate)
3. Increasing SF would decompress content that may exceed containment capacity
4. Decompression without sufficient containment is hypothesized to produce destabilization
5. Therefore: the system is expected to maintain compression as a stability-preserving operation

3.3 Predictions

- P1.1: Systems with low IRC will actively resist SF increases even when cognitively capable of higher-resolution processing
- P1.2: Resistance will be proportional to the gap between current IRC and the IRC required to contain higher-resolution content
- P1.3: Resistance will be domain-specific — strongest where higher resolution reveals the most destabilizing content

- P1.4: Resistance will manifest not as conscious refusal but as automatic defensive operations (projection, denial, devaluation of the source)

3.4 Distinction From Existing Accounts

Existing explanation	Mechanism proposed	SRF structural account
Motivational deficit	"Doesn't want to change"	Motivation may be present but structurally insufficient — the system may not be able to contain what change reveals
Cognitive limitation	"Can't see it"	May see it in low-risk domains — selectively cannot in high-risk ones
Characterological fixity	"Personality is stable"	Architecture is stable because instability would produce destabilization
Defense mechanism	"Unconscious protection"	Consistent with protection account, adds specification of the structural parameter (IRC) that determines necessity

3.5 Falsification

The hypothesis is falsified if: - SF increases produce no measurable destabilization under independently assessed low-IRC conditions (using proxy measures such as low distress tolerance scores, low ego strength, or poor affect regulation capacity — not inferred from resistance itself) - Resistance to SF increase shows no relationship to independently measured IRC proxies - Systems with independently assessed low IRC tolerate high-resolution content without destabilization

Note: falsification requires that IRC be operationalized through measures independent of the resistance phenomenon itself. Using resistance as evidence of low IRC and then predicting resistance from low IRC would be circular. Independent IRC proxies (distress tolerance scales, affect regulation measures, behavioral containment paradigms) are essential for non-circular testing.

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4. The Transitional Pain Hypothesis

4.1 Formal Statement

H2 — Transitional Pain: Increasing effective psychological resolution initially amplifies experienced distress by exposing contradiction, shame, dependency, and loss previously compressed by defensive simplification.

4.2 The J-Curve

The transition from low SF to higher SF follows a characteristic trajectory:

```
Distress
  ^
  |        *
  |      *   *
  |    *       *
  |   *          *
  |  *             *  *  *  *  (new baseline)
  | *
  |*  (old baseline)
  +-------------------------> Time
       ^           ^
       |           |
   SF increase   New coherence
    begins       established
```

Phase 1: Old baseline — low distress maintained through compression Phase 2: Decompression — previously split content becomes simultaneously available Phase 3: Peak distress — system holds contradictory content without yet having integrated it Phase 4: Integration — new coherence forms at higher resolution Phase 5: New baseline — lower distress than Phase 1 because coherence is reality-based rather than compression-based

4.3 Mechanism

1. At low SF, contradictory information is compressed — the system does not simultaneously represent incompatible self-states
2. As SF increases, previously compressed information decompresses — the system begins to simultaneously represent what was previously split
3. Decompression produces activation requiring containment:
   - Contradiction: "I am both good and harmful" (identity tension)
   - Shame: "I have caused damage I cannot undo" (shame tension)
   - Dependency: "I need others and cannot fully control this" (attachment tension)
   - Grief: "I have lost things that are gone" (loss activation)
4. This activation requires IRC to contain — it cannot be immediately processed
5. Only after sufficient processing time does the system achieve new coherence at higher resolution

4.4 Analogy: System Update Under Instability

The transition can be understood through an analogy to system updates: the system is temporarily less stable during transition than in either the pre-update or post-update state. Intermediate states lack the coherence of either configuration. The system is vulnerable during the transition window. Interrupting the process (premature scaffolding withdrawal) may leave the system less coherent than before.

This analogy illustrates the temporal structure of the claim — it is not intended as a literal mapping between software and psychological systems.

4.5 Predictions

- P2.1: Therapeutic interventions that increase SF will produce measurable distress amplification before improvement

- P2.2: The distress peak amplitude will correlate with the magnitude of resolution change
- P2.3: The distress peak duration will inversely correlate with available IRC
- P2.4: Systems with higher IRC will show shorter and lower-amplitude distress peaks
- P2.5: Premature SF increase (without adequate IRC) will produce fragmentation rather than the J-curve — distress rises and does not return to baseline

4.6 Distinction From Therapeutic Deterioration

The J-curve is not equivalent to therapeutic deterioration (Lambert & Ogles, 2004): - Deterioration implies treatment failure - The J-curve may reflect productive transitional destabilization under sufficient containment - The critical distinction: does distress resolve into higher-resolution coherence (J-curve) or persist/worsen without integration (deterioration)?

The framework proposes that the same intervention may produce the J-curve or deterioration depending on available IRC: sufficient IRC → transitional distress resolving into integration; insufficient IRC → destabilization without recovery. This prediction requires empirical testing — a distress spike alone does not confirm productive change.

4.7 Falsification

The hypothesis is falsified if: - SF increases produce monotonic improvement without transitional distress - The J-curve pattern shows no relationship to IRC level - Distress amplification during therapy is unrelated to resolution change

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5. The Bootstrap Problem

5.1 The Paradox

The interaction between H1 and H2 produces a structural paradox:

- Sustainable SF increase requires IRC >= IRC_crit (to contain decompressed content)
- Systems with chronically low SF are hypothesized to often co-occur with low IRC (the CED region)
- Therefore: self-generated structural change may be severely constrained

This is a bootstrap problem: capacity X is required to acquire capacity X.

5.2 Formal Structure

Let: - IRC_current = system's current holding threshold - IRC_crit(ΔSF) = minimum holding threshold required to contain the content revealed by SF increase ΔSF - SF_target = desired structural flexibility level

The system can sustainably increase SF only when:

IRC_current >= IRC_crit(ΔSF)

But in CED-region systems:

IRC_current < IRC_crit(ΔSF) for clinically relevant increases in resolution

The system is trapped: it cannot increase SF without holding capacity it does not have, and it cannot develop holding capacity without SF to process activation into stable structure. The severity of this constraint is expected to vary — it is most pronounced under severe IRC deficit and may be less absolute under moderate deficit where alternative developmental pathways (repeated exposure, pharmacological stabilization, gradual relational experience) may partially circumvent the containment requirement.

5.3 Severity of the Constraint

The bootstrap problem is not a statement about impossibility. It is a statement about severe constraint under self-generated conditions. If IRC develops primarily through successful containment experiences, and successful containment requires holding activation without discharge, and holding activation without discharge requires IRC — then self-generated structural change may be severely constrained.

This framing acknowledges that alternative pathways to IRC development may exist (repeated exposure, pharmacological stabilization, cognitive reframing, relational experience without explicit containment). The bootstrap argument does not claim these are impossible — it claims that the containment pathway, which may be the primary route to IRC development, is structurally self-limiting in low-IRC systems.

The term "functionally resistant under self-generated conditions" is therefore more precise than "immutable." The system is not permanently fixed — but the primary developmental pathway may be unavailable without external augmentation.

5.4 The Double Bind of Dependency

The bootstrap problem has a second layer specific to configurations with high externalization resistance:

- The solution to insufficient IRC is external scaffolding (co-regulation)
- Accepting co-regulation requires acknowledging dependency
- Acknowledging dependency is itself a high-resolution perception
- High-resolution perception requires the IRC the system lacks

The system resists the intervention that would enable change because accepting the intervention requires the change it would enable. This double bind is particularly pronounced in configurations where autonomy and self-sufficiency are central to the compressed self-representation.

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6. The Scaffolded Integration Hypothesis

6.1 Formal Statement

H3 — Scaffolded Integration: Sustainable integration requires temporary external holding structures that compensate for insufficient internal regulatory capacity during defensive destabilization.

6.2 Mechanism

1. The therapeutic relationship provides co-regulatory capacity — functioning as temporary augmentation of the system's holding threshold
2. This augmented threshold enables the system to tolerate transitional distress that would otherwise exceed its containment capacity
3. Under scaffolded conditions, SF can increase incrementally without fragmentation
4. As SF increases, the system develops its own IRC through successful processing experiences
5. Scaffolding becomes progressively less necessary as internal capacity develops

6. Eventually the system's IRC exceeds IRC_crit and scaffolding can be withdrawn

6.3 The Scaffolding Metaphor — And Its Limits

Architectural scaffolding is: - Structurally necessary during construction - External to the final structure - Removable once the structure is self-supporting - Not a permanent feature

Therapeutic scaffolding differs in one critical respect: the scaffolding itself (the therapeutic relationship) may become an object of attachment that the system must eventually tolerate losing — which itself requires IRC. The withdrawal of scaffolding is therefore not merely removal of support but a developmental challenge requiring the capacity the scaffolding helped build.

6.4 What Constitutes Adequate Scaffolding

The framework proposes that effective scaffolding provides:

Property	Function	Structural role
Consistency	Predictable availability	Reduces attachment activation during transition
Non-abandonment	Survival of the relationship under patient's externalization	Demonstrates that dependency does not destroy
Affect tolerance	Therapist contains what patient cannot yet contain	Functional augmentation of effective holding capacity
Pacing control	Regulates speed of SF increase	Prevents exceeding IRC_current + scaffolding
Non-retaliation	Absorbs externalization without counter-attack	Maintains safety during vector activation

6.5 Predictions

- P3.1: Therapeutic progress in low-IRC systems will correlate with the quality and stability of the holding environment
- P3.2: Premature withdrawal of scaffolding (before IRC reaches self-sustaining threshold) will produce regression
- P3.3: Systems that cannot tolerate therapeutic dependency will show poorer outcomes regardless of intervention technique
- P3.4: Minimum scaffolding duration will correlate with the magnitude of the IRC deficit
- P3.5: Scaffolded SF increase will produce the J-curve; unscaffolded SF increase in low-IRC systems will produce fragmentation or defensive reinforcement

6.6 Falsification

The hypothesis is falsified if: - Low-IRC systems show equivalent therapeutic outcomes with and without stable holding environments - Scaffolding duration shows no relationship to IRC development - Therapeutic relationship quality is unrelated to structural change in low-IRC populations

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7. The Domain-Selective Resolution Hypothesis

7.1 Formal Statement

H4 — Domain-Selective Resolution: Psychological resolution capacity operates domain-specifically, permitting higher-resolution processing in low-attachment contexts while maintaining defensive compression in attachment-relevant domains where fragmentation risk is greatest.

7.2 Mechanism

The proposed mechanism is that effective holding threshold varies by domain due to differential activation intensity in attachment-relevant and identity-relevant contexts:

1. Attachment-relevant and identity-relevant domains produce higher-intensity activation than instrumental domains when challenged
2. Higher-intensity activation requires greater holding capacity to contain
3. In domains where activation intensity is low (work, intellectual tasks), the system's existing IRC may be sufficient for higher-resolution processing
4. In domains where activation intensity is high (intimate relationships, self-evaluation under relational threat), the same IRC is insufficient, and compression is maintained

The framework does not claim that attachment is the only high-activation domain. Shame-relevant, status-relevant, and control-relevant domains may also produce elevated activation depending on individual developmental history. The prediction is that compression will be maintained preferentially in whichever domains produce the highest activation relative to available IRC.

7.3 Domain-Preserved Functioning Under Attachment-Specific Compression

This hypothesis provides a structural account of a well-known clinical observation: individuals who demonstrate sophisticated cognitive flexibility, professional competence, and nuanced judgment in work contexts while maintaining rigid dichotomous processing in intimate relationships.

The standard interpretation is that these individuals "choose" to be rigid in relationships or that their professional competence is "superficial." The structural interpretation is different: the system operates at the resolution its parameters permit in each domain. Professional contexts carry low destabilization risk (identity is not existentially threatened by a work disagreement), so higher resolution is structurally affordable. Intimate contexts carry high destabilization risk (identity coherence depends on the attachment relationship), so compression is structurally necessary.

7.4 Why This Matters Clinically

Domain-selective resolution may be the most directly observable prediction of the resolution resistance model. It generates a specific, testable pattern: the same individual will show measurably different SF levels across domains, and the difference will track activation intensity rather than cognitive ability.

This has immediate clinical implications: - Therapeutic gains appearing first in low-activation domains are not "superficial" — they reflect the structural logic of domain-selective compression - Expecting uniform resolution increase across all domains simultaneously may set unrealistic therapeutic goals - The domain gap itself may serve as a clinical indicator of IRC deficit magnitude

7.5 Predictions

- P4.1: Individuals with low overall SF will show measurably higher SF in low-attachment contexts than in high-attachment contexts
- P4.2: The domain-specificity gap will be proportional to the IRC deficit
- P4.3: Therapeutic gains in SF will generalize from low-risk to high-risk domains only after IRC reaches sufficient levels
- P4.4: Domain-selective resolution will be more pronounced in configurations with high attachment-specific ERD than in configurations with uniformly low SF
- P4.5: Under sufficient stress, even high-resolution domains will compress — the system sacrifices accuracy for coherence when IRC is depleted

7.6 Falsification

The hypothesis is falsified if: - SF shows no domain-specificity within individuals - Domain differences show no relationship to attachment-relevance or fragmentation risk - Professional competence and relational flexibility are always correlated

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8. Attractor Basin Interpretation

8.1 Heuristic Framework

The dynamics described in Sections 3–7 can be interpreted heuristically through attractor basin language from dynamical systems theory. This interpretation is offered as a conceptual lens that may aid intuition — not as a formal mathematical model. No state-space formalization is attempted here.

Each stable regulatory configuration can be understood as functioning analogously to an attractor basin — a region toward which the system gravitates and from which perturbation produces return rather than transition.

8.2 Basin Properties

Low SF (compressed dichotomous processing) functions analogously to a deep basin: high stability, low holding requirement, inaccurate but coherent, surrounded by painful territory.

Higher SF (integrated processing) functions analogously to a shallower basin: moderate stability, higher holding requirement, accurate and coherent, surrounded by less painful territory.

8.3 The Transition Landscape

The path between configurations passes through elevated activation — the content that decompression reveals (shame, grief, dependency, contradiction). The magnitude of this activation relative to the system's holding threshold determines whether transition is sustainable.

A system perturbed from the binary basin (e.g., by a moment of insight, a confrontation, a crisis) is expected to return to the basin unless the perturbation is sustained under sufficient holding capacity. Partial perturbation — enough to glimpse higher-resolution content but not enough to integrate it — may produce:

1. Momentary destabilization
2. Activation of shame, grief, or dependency

3. Exceeding of IRC threshold
4. Defensive re-compression (return to basin)
5. Often: reinforcement of the basin (the system "learns" that higher resolution is dangerous)

This may explain why insight alone rarely produces structural change. Insight is perturbation. Without sufficient IRC to sustain the transition, perturbation may produce return and reinforcement rather than lasting change.

8.4 Scaffolding as Threshold Augmentation

In this heuristic, therapeutic scaffolding can be understood as the combined effect of internal holding capacity and external co-regulatory contribution — functionally augmenting the system's effective threshold during transition. When this combined capacity exceeds the activation produced by decompression, transition becomes possible.

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9. Forced vs. Scaffolded Resolution Increase

9.1 Two Pathways

Two distinct pathways can produce SF increase:

Forced increase — Crisis, loss, unavoidable confrontation with reality: - Resolution increases without scaffolding - The system encounters high-resolution content without adequate external IRC - Outcome depends entirely on whether IRC_internal >= IRC_crit

Scaffolded increase — Therapeutic relationship, stable co-regulation: - Resolution increases within a holding environment - External IRC compensates for internal deficit - Incremental progression possible - Lower fragmentation risk

9.2 Forced Increase: Divergent Outcomes

When resolution is forced (crisis, loss, inescapable evidence), the framework predicts two broad outcome classes:

If IRC_internal >= IRC_crit: - Painful but potentially integrative - The system contains the decompressed content - New coherence may form at higher resolution - Consistent with post-traumatic growth literature

If IRC_internal < IRC_crit: - Destabilization without integration - The system cannot contain the decompressed content - Two possible outcomes: - Acute self-state fragmentation: loss of coherent self-state, decompensation - Defensive reinforcement: the system rebuilds compressed dichotomous processing at a deeper level, with stronger resistance to future perturbation

9.3 Scaffolded Increase: Gradual Trajectory

Scaffolded increase is hypothesized to produce lower outcome variance: - Incremental resolution steps (binary → low → moderate → high) - Each step requires only the IRC sufficient for that increment - Scaffolding compensates for IRC deficit at each step - Lower variance in outcomes - But: requires tolerance of dependency (the double bind of Section 5.4)

9.4 Predictions

- P5.1: Forced resolution increases are predicted to produce divergent outcome distributions (integration vs. destabilization/reinforcement)
- P5.2: The probability of integration under forced increase is expected to correlate with pre-crisis IRC level
- P5.3: Scaffolded increases will produce unimodal, gradual improvement with lower outcome variance
- P5.4: Defensive reinforcement following failed forced increase will manifest as increased resistance to subsequent change attempts

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10. Minimum Viable Resolution Increment

Note: This section outlines a secondary prediction that follows from the primary mechanism. It is included for completeness but may warrant fuller treatment in future work.

10.1 The Granularity Question

Must therapeutic change proceed from binary processing directly to full integration? Or can the system transition through intermediate resolutions? This section briefly outlines the possibility of incremental change — a topic warranting fuller treatment in future work.

10.2 Incremental Steps

The framework proposes that resolution increase can proceed incrementally:

Binary → Low-resolution: "This person is mostly good" (without yet "and I am partly responsible") - Requires: minimal IRC increase - Reveals: partial complexity of other - Does not yet reveal: self-implication

Low-resolution → Moderate-resolution: "I am partly good, partly inadequate; they are partly right" - Requires: moderate IRC increase - Reveals: self-implication, partial responsibility - Does not yet reveal: full grief, full dependency

Moderate-resolution → High-resolution: Full ambivalence, responsibility, dependency, grief held simultaneously - Requires: substantial IRC - Reveals: complete experiential complexity

10.3 Clinical Implication

Each increment requires only the IRC sufficient for that step. This suggests that therapeutic pacing should match the system's current IRC — pushing for the next increment rather than demanding full integration.

The minimum viable increment is the smallest resolution increase that: 1. Produces meaningful experiential change 2. Does not exceed current IRC + scaffolding 3. Can be consolidated before the next increment

10.4 Predictions

- P6.1: Smaller resolution increments will require less IRC and produce lower-amplitude J-curves
- P6.2: Systems will show stable plateaus at intermediate resolutions before progressing further
- P6.3: Attempting to skip increments (binary → moderate without low-resolution consolidation) may produce higher destabilization risk than sequential progression

11. Relationship to Existing Change Models

11.1 Dabrowski's Positive Disintegration

Dabrowski (1966) proposed that psychological development requires disintegration of existing structures before reintegration at higher levels. The present framework is structurally consistent with Dabrowski but adds: - Specification of the parameter (IRC) that determines whether disintegration leads to reintegration or collapse - The bootstrap problem (why some systems cannot initiate disintegration) - Domain-selectivity (disintegration may be domain-specific rather than global)

11.2 Stages of Change (Prochaska & DiClemente)

The transtheoretical model describes stages (precontemplation, contemplation, preparation, action, maintenance) without specifying the structural mechanism that determines stage transitions. The present framework offers one possible interpretation: stage transitions may be interpretable through IRC threshold language — precontemplation as IRC < IRC_crit (change structurally constrained); contemplation as IRC approaching IRC_crit; action as IRC >= IRC_crit. This reinterpretation is speculative and requires empirical testing.

11.3 Therapeutic Alliance Research

Decades of research demonstrate that therapeutic alliance predicts outcomes across modalities (Horvath et al., 2011; Flückiger et al., 2018). The present framework offers one possible structural interpretation: alliance may provide scaffolding (temporary holding-capacity augmentation) that enables resolution increase. If correct, this would offer a mechanism for the well-established but theoretically underspecified alliance–outcome relationship. This interpretation does not claim to be the only or complete explanation.

11.4 Winnicott's Holding Environment

Winnicott's (1960) concept of the holding environment is the closest existing formulation to the scaffolding hypothesis. The present framework attempts to extend Winnicott by: - Specifying what is being held (decompressed content that may exceed IRC) - Proposing a quantifiable holding requirement (IRC_crit - IRC_current) - Predicting when holding can be withdrawn (when IRC_internal >= IRC_crit) - Offering a structural account of why some systems resist holding (the double bind of dependency)

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12. Measurement Directions

12.1 J-Curve Detection

- Repeated distress measures during therapeutic SF interventions
- Testing for initial amplification followed by reduction
- Moderation analysis by IRC level
- Distinguishing J-curve from deterioration through trajectory shape

12.2 IRC Threshold Identification

- Experimental paradigms incrementally increasing processing demands
- Monitoring coherence indicators (self-concept clarity, identity stability)
- Identifying the demand level at which coherence breaks down
- Individual differences in threshold as a function of baseline IRC

12.3 Domain-Specificity Assessment

- Cross-domain SF measurement within individuals
- Comparing SF in professional vs. intimate contexts
- Testing whether domain gap correlates with IRC deficit
- Longitudinal tracking of domain generalization during therapy

12.4 Scaffolding Quality Measurement

- Operationalizing therapeutic holding environment properties
- Relating holding quality to structural change outcomes
- Testing whether scaffolding quality moderates the J-curve

12.5 Resolution Increment Tracking

- Measuring effective resolution level across therapy
- Testing for discrete steps vs. continuous change
- Identifying plateau durations between increments

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13. Limitations

13.1 IRC_crit Is Unspecified

The framework proposes a critical IRC threshold but does not quantify it. Whether a universal threshold exists or whether it varies by individual, domain, and resolution increment remains unknown.

13.2 J-Curve Parameters Are Unspecified

The amplitude, duration, and shape of transitional distress are proposed but not parameterized. These require empirical characterization.

13.3 Scaffolding Operationalization

What constitutes adequate therapeutic scaffolding is described conceptually but not operationally defined. Different therapeutic traditions may provide scaffolding through different mechanisms.

13.4 Domain Boundaries Are Unclear

The framework proposes domain-selective resolution but does not specify how domains are delineated or how many functionally distinct domains exist.

13.5 Circularity Risk

The bootstrap argument risks circularity: it assumes IRC develops primarily through containment, then concludes that containment is impossible without IRC. This circularity is acknowledged. The argument's validity depends on the empirical question of whether containment is indeed the primary pathway to IRC development. If alternative pathways (pharmacological stabilization, repeated exposure, cognitive reframing, relational experience without explicit containment) prove equally effective, the bootstrap constraint is weaker than proposed. Independent measurement of IRC through non-resistance-based indicators is essential.

13.6 Overlap With Existing Models

The J-curve concept overlaps with existing models of therapeutic deterioration, stages of change, and positive disintegration. The framework must demonstrate structural specificity beyond these existing formulations.

13.8 Analogical Status

This paper uses structural analogies (signal processing, attractor basins, system updates) as explanatory tools to generate testable hypotheses. These analogies are heuristic — they illuminate structural similarities between psychological regulation and well-understood physical systems, but they are not ontological claims. The psychological system is not literally a signal processor or a dynamical system in the mathematical sense. The value of the analogies lies in the predictions they generate, not in their literal truth.

13.9 Empirical Status

All hypotheses in this paper are theoretical. No empirical validation exists. The paper's value is in generating testable predictions, not in reporting findings.

13.10 Dependency on SRF Constructs

This paper's arguments depend entirely on the validity of SRF constructs (IRC, SF, ERD) as defined in Halmetoja (2026). If these constructs prove empirically indistinguishable from existing measures (distress tolerance, mentalization, attachment dependence), the present paper's claims reduce to restatements of known phenomena in novel terminology. The paper's independent contribution is contingent on SRF constructs demonstrating incremental validity — particularly the threshold framing of IRC and the domain-specificity of SF.

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14. Summary: Primary Mechanism and Derived Predictions

Primary Mechanism

Resolution Resistance: Defensive simplification (low SF) persists as an active stability-preserving operation under insufficient holding capacity (IRC). The system maintains compression because decompression would exceed its structural containment threshold.

Derived Predictions

Derived prediction	Relationship to primary mechanism	Key testable claim	Falsified if
Transitional Pain	Consequence of resolution increase	J-curve trajectory moderated by IRC	SF increase produces monotonic improvement
Scaffolded Integration	Solution to the bootstrap problem	Holding quality predicts outcomes in low-IRC systems	Outcomes unrelated to holding environment
Domain-Selective Resolution	Expression of selective compression	Higher SF in low-risk than high-risk domains	No domain-specificity in SF

The derived predictions are not independent hypotheses — they follow logically from the primary mechanism under different conditions (during change, enabling change, and across contexts respectively).

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15. Future Directions

15.1 Immediate Priorities

1. Development of domain-specific SF measures
2. Longitudinal J-curve detection studies
3. IRC threshold identification paradigms

15.2 Longer-Term Research

4. Scaffolding quality operationalization and outcome prediction
5. Minimum viable increment identification
6. Forced vs. scaffolded outcome comparison studies
7. Integration with neurophysiological measures (Ne, HRV) from SRF Paper 1

15.3 Clinical Translation

8. Therapeutic pacing guidelines based on IRC assessment
9. Scaffolding withdrawal timing protocols
10. Domain-selective intervention sequencing

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